

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction for the administration of Flucloxacillin for the treatment of Skin Infections. If this is unsuitable, or the person has a penicillin allergy, for the administration of Clarithromycin or Doxycycline for the treatment of skin infections.

Summary of NICE / NICE CKS Guidelines for Skin Infections

1. Impetigo

Overview

- **Superficial bacterial skin infection**, commonly caused by *Staphylococcus aureus* or *Streptococcus pyogenes*.
- Non-bullous (crusted) and bullous types.

Treatment

- **Topical first-line** for localised, non-bullous cases:
 - **Hydrogen peroxide 1% cream** (first-line if limited lesions).
 - **Fusidic acid 2% cream** (short course if hydrogen peroxide unsuitable).
- **Oral antibiotics** for extensive or systemically unwell:
 - **Flucloxacillin** (first-line).
 - **Clarithromycin or erythromycin** (penicillin allergy).

2. Cellulitis and Erysipelas

Overview

- Acute bacterial infection of the **dermis and subcutaneous tissue** (cellulitis) or **upper dermis/lymphatics** (erysipelas).
- Usually caused by *Streptococcus pyogenes* or *S. aureus*.
- Erysipelas has well-defined, raised margins; cellulitis is more diffuse.

Treatment

- **Flucloxacillin 500 mg QDS for 5–7 days** (first-line).
- **Clarithromycin or doxycycline** if penicillin-allergic.
- **IV antibiotics** if systemically unwell or severe infection.

Mark skin margins to monitor spread and response.

3. Boils (Furuncles) and Carbuncles

Overview

- **Infection of hair follicle** leading to abscess formation (furuncle = single; carbuncle = cluster).
- Typically caused by *S. aureus*.

Management

- **Warm compresses** for small, uncomplicated lesions.
 - **Incision and drainage** for fluctuant or large abscesses.
 - **Oral antibiotics** (e.g. flucloxacillin) only if:
 - Surrounding cellulitis
 - Multiple/recurrent lesions
 - Systemic symptoms
 - Immunosuppressed
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4. Infected Eczema or Dermatitis

Signs of Infection

- Weeping, crusting, worsening inflammation, or failure to respond to emollients/steroids.

Treatment

- Continue topical corticosteroids.
 - Add **topical fusidic acid** or **oral flucloxacillin** if widespread/systemic signs.
 - Avoid long-term or repeated topical antibiotics due to resistance risk.
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General Management Principles

- Clean affected area with soap and water where appropriate.
 - Avoid topical antibiotics for more than **5–7 days**.
 - Encourage good skin hygiene.
 - Consider MRSA in recurrent or treatment-resistant cases.
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When to Refer

- Severe or recurrent infections
- Unclear diagnosis
- Rapidly spreading infection
- Immunocompromised patients
- Suspected **necrotising fasciitis** (urgent)

Patient Group Direction

for the supply/administration of

Flucloxacillin

for the treatment of

Skin Infections

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Flucloxacillin is indicated for the treatment of mild to moderate skin and soft tissue infections, such as impetigo, folliculitis, infected eczema, and infected wounds in adults and children aged 2 years and over.
Inclusion criteria	- Adults and children aged 2 years and over. - Presenting with signs of mild to moderate skin infection where flucloxacillin is appropriate. - Informed consent obtained from patient or guardian. - No contraindications to flucloxacillin.
Exclusion criteria	- Known hypersensitivity to flucloxacillin, penicillins, or beta-lactam antibiotics. - Severe systemic infection requiring intravenous therapy. - History of flucloxacillin-associated jaundice or hepatic dysfunction. - Pregnancy - Breastfeeding individuals unless under appropriate supervision. - Children under 2 years of age. - Immunosuppression - Taking any medication with a clinically significant interaction. - Severe renal failure with creatinine clearance less than 10ml/min
Cautions	- Administer before food for optimal absorption. - Use with caution in patients with renal impairment or history of hepatic issues. - Complete full course even if symptoms resolve earlier. - Refer if symptoms worsen or do not improve within 2-3 days. - Advise patients to complete the full course and report any side effects. - Consider <i>Clostridioides difficile</i> risk in patients with recent antibiotic use or hospitalisation. - Caution is advised when flucloxacillin is administered concomitantly with paracetamol due to the increased risk of high anion gap metabolic acidosis (HAGMA). Patients at particularly high risk are those with severe renal impairment, sepsis or malnutrition, especially if the maximum daily doses of paracetamol are used. After co-administration of flucloxacillin and paracetamol, a close monitoring is recommended in order to detect the appearance of acid-base disorders, namely HAGMA. - The occurrence at the treatment initiation of a feverish generalised erythema associated with pustula may be a symptom of acute generalised exanthematous pustulosis (AGEP). In case of AGEP diagnosis, flucloxacillin should be discontinued and any subsequent administration of flucloxacillin contra-indicated. - The use of flucloxacillin (like other penicillins) in patients with renal impairment does not usually require dosage reduction. In the presence of severe renal failure (creatinine clearance less than 10ml/min), however, a reduction in dose or an extension of dose interval should be

	considered because of the risk of neurotoxicity and such patients are excluded from the PGD. - Hepatitis and cholestatic jaundice have been reported. These reactions are related neither to the dose nor to the route of administration. - Flucloxacillin should be used with caution in patients with evidence of hepatic dysfunction (excluded from PGD), patients over 50 years or patients with underlying disease all of whom are at increased risk of hepatic reactions. The onset of these hepatic effects may be delayed for up to two months post-treatment. In several cases, the course of the reactions has been protracted and lasted for some months. In very rare cases, a fatal outcome has been reported. - As for other penicillins contact with the skin should be avoided as sensitisation may occur. - Patients with a known history of allergy are more likely to develop a hypersensitivity reaction. - Prolonged use of an anti-infective agent may occasionally result in overgrowth of non-susceptible organisms. - Before initiating therapy with flucloxacillin, careful enquiry should be made concerning previous hypersensitivity reactions to β-lactams. Cross-sensitivity between penicillins and cephalosporins is well documented. Serious and occasionally fatal hypersensitivity reactions (anaphylaxis) have been reported in patients receiving β-lactam antibiotics. Although anaphylaxis is more frequent following parenteral therapy, it has occurred in patients on oral therapy. These reactions are more likely to occur in individuals with a history of β-lactam hypersensitivity. If anaphylaxis occurs flucloxacillin should be discontinued and the appropriate therapy instituted. Serious anaphylactic reactions may require immediate emergency treatment with adrenaline (epinephrine). Ensure adequate airway and ventilation and give 100% oxygen. IV crystalloids, hydrocortisone, antihistamine and nebulised bronchodilators may also be required. - Hypokalaemia (potentially life threatening) can occur with the use of flucloxacillin, especially in high doses. Hypokalaemia caused by flucloxacillin can be resistant to potassium supplementation. Regular measurements of potassium levels are recommended during the therapy with higher doses of flucloxacillin. Attention for this risk is warranted also when combining flucloxacillin with hypokalemia-inducing diuretics or when other risk factors for the development of hypokalemia are present (e.g. malnutrition, renal tubule dysfunction).
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Flucloxacillin 250mg or 500mg capsules or 250mg/5ml suspension
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Legal category	POM
Storage	Capsules: Store below 25°C in a dry place. Suspension: Store in a refrigerator (2°C -8°C)
Route/method of administration	Oral administration on an empty stomach. Flucloxacillin capsules or suspension should be taken at least 1 hour before or 2 hours after food. They should be taken with a full glass of water (250 ml) to reduce the risk of oesophageal pain and patients should not lay down immediately after Flucloxacillin intake.
Dose and frequency	Child 2-9 years: 125–250 mg 4 times a day for 5–7 days. Child 10–17 years: 250–500 mg 4 times a day for 5–7 days. Adult: 500 mg 4 times a day for 5–7 days.
Quantity to be administered and/or supplied	Capsules: 20 or 28 capsules depending on clinical judgement. Suspension: 100 or 200ml depending on clinical judgement.
Maximum or minimum treatment period	Typically 5 to 7 days depending on infection severity and response.
Adverse effects	Nausea, diarrhoea, abdominal pain, skin rash. Rare: cholestatic jaundice, anaphylaxis. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
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Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 2-3 days, or they become systemically very unwell. Advising the person on hygiene measures to aid healing and stop the spread of the infection. Ensuring pre-existing skin conditions (such as eczema) are optimally treated. Take paracetamol or ibuprofen for pain and fever and drink adequate fluids. Advise that the skin may take some time to return to normal after the course of antibiotics has finished.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
4/1/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		4/1/26
Chris Pilkington	Pharmacist GPhC: 2046322		4/1/26

Change history

Version number	Change details	Date
001	Development & issue of new PGD	4 th Jan 2026

Patient Group Direction

for the supply/administration of

Clarithromycin

for the treatment of

Skin infection

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Flucloxacillin is indicated for the treatment of mild to moderate skin and soft tissue infections, such as impetigo, folliculitis, infected eczema, and infected wounds in adults and children aged 2 years and over.
Inclusion criteria	- Adults and children aged 2 years and over. - Presenting with signs of mild to moderate skin infection where clarithromycin is appropriate. - No known allergy to macrolides. - Informed consent obtained from patient or guardian. - No contraindications to clarithromycin
Exclusion criteria	- Severe systemic infection requiring intravenous therapy. - Children under 2 years of age. - Known allergy or hypersensitivity to macrolides - Signs of severe infection (e.g. systemic involvement, sepsis). - Abscess requiring drainage or infected wound needing surgical review. - Pregnant or breastfeeding individuals - Hepatic dysfunction or history - Clarithromycin is contraindicated in patients with known hypersensitivity to the active substance clarithromycin, to other macrolide antibiotic drugs or to any of the excipients. - Immunosuppression. - Taking any medication with a clinically significant interaction. - Concomitant administration of clarithromycin and ergot alkaloids (e.g., ergotamine or dihydroergotamine) is contraindicated, as this may result in ergot toxicity. - Concomitant administration of clarithromycin and oral midazolam is contraindicated. - Concomitant administration of clarithromycin and lomitapide is contraindicated. - Concomitant administration of clarithromycin and any of the following drugs is contraindicated: astemizole, cisapride, domperidone, pimozone and terfenadine as this may result in QT prolongation and cardiac arrhythmias, including ventricular tachycardia, ventricular fibrillation and torsades de pointes. - Clarithromycin should not be given to patients with history of QT prolongation (congenital or documented acquired QT prolongation) or ventricular cardiac arrhythmia, including torsades de pointes. - Concomitant administration with ticagrelor, ivabradine or ranolazine is contraindicated. - Clarithromycin should not be used concomitantly with HMG-CoA reductase inhibitors (statins) that are extensively metabolised by CYP3A4 (lovastatin or simvastatin), due to the increased risk of myopathy, including rhabdomyolysis.

	- As with other strong CYP3A4 inhibitors, Clarithromycin should not be used in patients taking colchicine. - Clarithromycin should not be given to patients with electrolyte disturbances (hypokalaemia or hypomagnesaemia, due to the risk of prolongation of the QT interval). - Clarithromycin should not be used in patients who suffer from severe hepatic failure in combination with renal impairment.
Cautions	- Complete full course even if symptoms resolve earlier. - Refer if symptoms worsen or do not improve within 7 days. - Advise patients to complete the full course and report any side effects. - Consider <i>Clostridioides difficile</i> risk in patients with recent antibiotic use or hospitalisation. - Clarithromycin is principally metabolised by the liver. Therefore, caution should be exercised in administering this antibiotic to patients with impaired hepatic function. - Caution should also be exercised when administering clarithromycin to patients with moderate to severe renal impairment - Hepatic dysfunction, including increased liver enzymes, and hepatocellular and/or cholestatic hepatitis, with or without jaundice, has been reported with clarithromycin. This hepatic dysfunction may be severe and is usually reversible. Cases of fatal hepatic failure have been reported. Some patients may have had pre-existing hepatic disease or may have been taking other hepatotoxic medicinal products. Patients should be advised to stop treatment and contact their doctor if signs and symptoms of hepatic disease develop, such as anorexia, jaundice, dark urine, pruritus, or tender abdomen. - Pseudomembranous colitis has been reported with nearly all antibacterial agents, including macrolides, and may range in severity from mild to life-threatening. <i>Clostridioides difficile</i>-associated diarrhoea (CDAD) has been reported with use of nearly all antibacterial agents including clarithromycin, and may range in severity from mild diarrhoea to fatal colitis. Treatment with antibacterial agents alters the normal flora of the colon, which may lead to overgrowth of <i>C. difficile</i>. CDAD must be considered in all patients who present with diarrhoea following antibiotic use. Careful medical history is necessary since CDAD has been reported to occur over two months after the administration of antibacterial agents. Therefore, discontinuation of clarithromycin therapy should be considered regardless of the indication. Microbial testing should be performed and adequate treatment initiated. Drugs inhibiting peristalsis should be avoided. - There have been post-marketing reports of colchicine toxicity with concomitant use of clarithromycin and colchicine, especially in the elderly, some of which occurred in patients with renal insufficiency. Deaths have been reported in some such patients. Concomitant administration of clarithromycin and colchicine is contraindicated.

	- Caution is advised regarding concomitant administration of clarithromycin and triazolobenzodiazepines, such as triazolam, and intravenous or oromucosal midazolam. - Cardiovascular Events: Prolongation of the QT interval, reflecting effects on cardiac repolarisation imparting a risk of developing cardiac arrhythmia and torsades de pointes, have been seen in patients treated with macrolides including clarithromycin. Due to increased risk of QT prolongation and ventricular arrhythmias (including torsades de pointes), the use of clarithromycin is contraindicated in patients taking any of astemizole, cisapride, domperidone, pimozone and terfenadine; in patients who have electrolyte disturbances such as hypomagnesaemia or hypokalaemia; and in patients with a history of QT prolongation or ventricular cardiac arrhythmia. Epidemiological studies investigating the risk of adverse cardiovascular outcomes with macrolides have shown variable results. Some observational studies have identified a rare short-term risk of arrhythmia, myocardial infarction and cardiovascular mortality associated with macrolides including clarithromycin. Consideration of these findings should be balanced with treatment benefits when prescribing clarithromycin. - Pneumonia: In view of the emerging resistance of Streptococcus pneumoniae to macrolides, it is important that sensitivity testing be performed when prescribing clarithromycin for community-acquired pneumonia. In hospital-acquired pneumonia, clarithromycin should be used in combination with additional appropriate antibiotics. - Skin and soft tissue infections of mild to moderate severity: These infections are most often caused by Staphylococcus aureus and Streptococcus pyogenes, both of which may be resistant to macrolides. Therefore, it is important that sensitivity testing be performed. In cases where beta-lactam antibiotics cannot be used (e.g. allergy), other antibiotics, such as clindamycin, may be the drug of first choice. Currently, macrolides are only considered to play a role in some skin and soft tissue infections, such as those caused by Corynebacterium minutissimum, acne vulgaris, and erysipelas and in situations where penicillin treatment cannot be used. - In the event of severe acute hypersensitivity reactions, such as anaphylaxis, severe cutaneous adverse reactions (SCAR) (e.g. Acute generalised exanthematous pustulosis (AGEP), Stevens-Johnson Syndrome, toxic epidermal necrolysis and Drug Rash with Eosinophilia and Systemic Symptoms (DRESS), clarithromycin therapy should be discontinued immediately and appropriate treatment should be urgently initiated. - Clarithromycin should be used with caution when administered concurrently with medications that induce the cytochrome CYP3A4 enzyme.
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	- Carefully consider the balance of benefits and risks before prescribing clarithromycin for any patients taking hydroxychloroquine or chloroquine, because of the potential for an increased risk of cardiovascular events and cardiovascular mortality. Furthermore, clarithromycin should be used with caution in the following: • Patients with coronary artery disease, severe cardiac insufficiency, conduction disturbances or clinically relevant bradycardia; • Patients concomitantly taking other medicinal products associated with QT prolongation other than those which are contraindicated - Oral hypoglycaemic agents/Insulin: The concomitant use of clarithromycin and oral hypoglycaemic agents (such as sulfonylurias) and/or insulin can result in significant hypoglycaemia. Careful monitoring of glucose is recommended. - Oral anticoagulants: There is a risk of serious haemorrhage and significant elevations in International Normalized Ratio (INR) and prothrombin time when clarithromycin is co-administered with warfarin. INR and prothrombin times should be frequently monitored while patients are receiving clarithromycin and oral anticoagulants concurrently. - Caution should be exercised when clarithromycin is co-administered with direct acting oral anticoagulants such as dabigatran, rivaroxaban, apixaban and edoxaban, particularly to patients at high risk of bleeding. - Long-term use may, as with other antibiotics, result in colonisation with increased numbers of non-susceptible bacteria and fungi. If superinfections occur, appropriate therapy should be instituted. - Attention should also be paid to the possibility of cross resistance between clarithromycin and other macrolide drugs, as well as lincomycin and clindamycin.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Clarithromycin tablets 250mg or 500mg. Clarithromycin suspension 250mg/5ml
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	By oral administration, twice daily for 5 to 7 days
Dose and frequency	Child 2 years –11 years (body-weight up to 8 kg): 7.5 mg/kg twice daily.

	Child 2 years –11 years (body-weight 8–11 kg): 62.5 mg twice daily. Child 2 years –11 years (body-weight 12–19 kg): 125 mg twice daily. Child 2 years –11 years (body-weight 20–29 kg): 187.5 mg twice daily. Child 2 years –11 years (body-weight 30–40 kg): 250 mg twice daily. Child 12–17 years: 250 mg twice daily usually for 5-7 days, alternatively 500 mg twice daily usually for 5-7days. Adult: 250 mg twice daily usually for 5-7 days, alternatively 500 mg twice daily usually for 5-7 days, increased dose used if required in severe infections. In patients with renal impairment with creatinine clearance less than 30 mL/min, the dosage of clarithromycin should be reduced by one-half, 250 mg twice daily in more severe infections.
Quantity to be administered and/or supplied	Tablets: 10 tablets for 5 days or 14 tablets for 7 days. Suspension: 70ml for 5 days or 140ml for 7 days
Maximum or minimum treatment period	Typically, 5 to 7 days depending on severity and response.
Adverse effects	The most frequent and common adverse reactions related to clarithromycin therapy are abdominal pain, diarrhoea, nausea, vomiting and taste perversion. Refer to SmPC for full details.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
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Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 2-3 days, or they become systemically very unwell. Advising the person on hygiene measures to aid healing and stop the spread of the infection. Ensuring pre-existing skin conditions (such as eczema) are optimally treated. Take paracetamol or ibuprofen for pain and fever and drink adequate fluids. Advise that the skin may take some time to return to normal after the course of antibiotics has finished.

Key references

• NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 • NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources • NICE CKS Guidance • British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) NICE • Summary of Product Characteristics for drug
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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
4/1/26		31/10/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		4/1/26
Chris Pilkington	Pharmacist GPhC: 2046322		4/1/26

Change history

Version number	Change details	Date
001	Development & issue of new PGD	4 th Jan 2026

Patient Group Direction

for the supply/administration of

Doxycycline caps 100mg

for the treatment of

Skin infections

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Doxycycline is indicated for the treatment of mild to moderate skin and soft tissue infections, such as impetigo, folliculitis, infected eczema, and infected wounds in adults and children aged 12 years and over.
Inclusion criteria	- Adults and children aged 12 years and over. - Presenting with signs of mild to moderate skin infection where doxycycline is appropriate. - Informed consent obtained from patient or guardian. - No contraindications to doxycycline. - No known allergy to doxycycline or other tetracyclines
Exclusion criteria	- Children under 12 years of age. - Immunosuppression - Taking any medication with a clinically significant interaction. - Known allergy or hypersensitivity to doxycycline or other tetracycline antibiotics. - Signs of severe infection (e.g. systemic involvement, sepsis). - Abscess requiring drainage or infected wound needing surgical review. - Pregnancy or breastfeeding. - Taking any medication with a clinically significant interaction.
Cautions	- Complete full course even if symptoms resolve earlier. - Refer if symptoms worsen or do not improve within 2-3 days. - Advise patients to complete the full course and report any side effects. - Use with caution in patients with renal impairment or history of hepatic issues (or concurrently receiving potentially hepatotoxic drugs). - Advise patients to complete the full course and report any side effects. - Consider <i>Clostridioides difficile</i> risk in patients with recent antibiotic use or hospitalisation. - Photosensitivity manifested by an exaggerated sunburn reaction has been observed in some individuals taking tetracyclines, including doxycycline. Patients likely to be exposed to direct sunlight or ultraviolet light should be advised that this reaction can occur with tetracycline drugs and treatment should be discontinued at the first evidence of skin erythema. - Photoonycholysis has also been reported in patients receiving doxycycline. - Benign intracranial hypertension (pseudotumor cerebri) has been associated with the use of tetracyclines including doxycycline. Benign intracranial hypertension (pseudotumor cerebri) is usually transient, however cases of permanent visual loss secondary to benign intracranial hypertension (pseudotumor cerebri) have been reported with tetracyclines including doxycycline. If visual disturbance occurs during treatment, prompt ophthalmologic evaluation is warranted. Since intracranial pressure can remain elevated for weeks after drug cessation patients should be monitored until they stabilize. Concomitant use of

	isotretinoin or other systemic retinoids and doxycycline should be avoided because isotretinoin is also known to cause benign intracranial hypertension (pseudotumor cerebri). - Microbiological overgrowth The use of antibiotics may occasionally result in the overgrowth of non-susceptible organisms including Candida. If a resistant organism appears, the antibiotic should be discontinued and appropriate therapy instituted. Pseudomembranous colitis has been reported with nearly all antibacterial agents, including doxycycline, and has ranged in severity from mild to life-threatening. It is important to consider this diagnosis in patients who present with diarrhoea subsequent to the administration of antibacterial agents. Clostridium difficile associated diarrhoea (CDAD) has been reported with use of nearly all antibacterial agents, including doxycycline, and may range in severity from mild diarrhoea to fatal colitis. Treatment with antibacterial agents alters the normal flora of the colon leading to overgrowth of C. difficile. C. difficile produces toxins A and B which contribute to the development of CDAD. Hypertoxin producing strains of C. difficile cause increased morbidity and mortality, as these infections can be refractory to antimicrobial therapy and may require colectomy. CDAD must be considered in all patients who present with diarrhoea following antibiotic use. Careful medical history is necessary since CDAD has been reported to occur over two months after the administration of antibacterial agents. -Oesophagitis Instances of oesophagitis and oesophageal ulcerations have been reported in patients receiving capsule and tablet forms of drugs in the tetracycline class, including doxycycline. Most of these patients took medications immediately before going to bed or with inadequate amounts of fluid. -Porphyria There have been rare reports of porphyria in patients receiving tetracyclines. -Myasthenia gravis Due to a potential for weak neuromuscular blockade, care should be taken in administering tetracyclines to patients with myasthenia gravis. -Systemic lupus erythematosus Tetracyclines can cause exacerbation of SLE. -Methoxyflurane Caution is advised in administering tetracyclines with methoxyflurane. -Alcohol dependence — alcohol may decrease the half-life of doxycycline.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Doxycycline 100mg capsules
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	Oral administration, the capsules should be swallowed with a full glass of water (250ml) in either the sitting or standing position and well before retiring at night to reduce the risk of oesophageal irritation and ulceration. If gastric irritation occurs, it is recommended that Doxycycline capsules be given with food or milk. Studies indicate that the absorption of doxycycline is not notably influenced by simultaneous ingestion of food or milk.
Dose and frequency	200mg on the first day and then 100mg daily. Usual course length of 5 to 7 days depending on clinical judgement.
Quantity to be administered and/or supplied	6 capsules for 5 days and 8 capsules for 7 days.
Maximum or minimum treatment period	Typically, 5 to 7 days depending on severity and response.
Adverse effects	Refer to SmPC for further detail

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
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Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 2-3 days, or they become systemically very unwell. Advising the person on hygiene measures to aid healing and stop the spread of the infection. Ensuring pre-existing skin conditions (such as eczema) are optimally treated. Take paracetamol or ibuprofen for pain and fever and drink adequate fluids. Advise that the skin may take some time to return to normal after the course of antibiotics has finished.

Key references

● NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 ● NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources ● NICE CKS Guidance ● British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) NICE ● Summary of Product Characteristics for drug
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Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
4/1/26		31/10/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		4/1/26
Chris Pilkington	Pharmacist GPhC: 2046322		4/1/26

Change history

Version number	Change details	Date
001	Development & issue of new PGD	4 th Jan 2026